

# Chronic Heart Failure Management — Clinical One-Pager

Classification · HFrEF 4-pillar GDMT · titration · EF spectrum. Core per 2022 AHA/ACC/HFSA. Adapted from TeachIM.org (CC BY-NC 4.0). Educational use only.

## 1 · The HF one-liner

**NYHA** (I asx · II sx w/ normal activity · III sx w/ less-than-ordinary · IV at rest/ADLs) + **EF** + **etiology** (ischemic ~50-60% vs non-ischemic: HTN, EtOH, tachy-induced, infiltrative, genetic).

**EF**: HFrEF ≤40 · HFmrEF 41-49 · HFpEF ≥50 · HFimpEF = was ≤40, improved ≥10 pts to >40.

## Roadblocks — keep GDMT going

**AKI**: a <30% GFR dip on ACEi/ARNI + SGLT2i is expected & OK; fix volume/diuretic first, hold ACEi/ARNI temporarily, re-trial after resolution.

**Low BP**: treat symptoms not the number (asx SBP 90-100 OK); drop non-GDMT antiHTN (CCB/thiazide); stagger dosing; carvedilol→metop succinate.

**HyperK**: SGLT2i & ARNI mitigate; low-K diet; hydralazine/isosorbide as last resort.

## 2 · HFrEF 4-pillar GDMT

| Class       | Agent · target                                                 | NNT |
|-------------|----------------------------------------------------------------|-----|
| BB          | metop <b>succinate</b> 200 / carvedilol 25-50 BID / bisoprolol | 9   |
| ARNI (pref) | sacubitril-valsartan 97/103 BID                                | 27  |
| ACEi/ARB    | lisinopril 40 / losartan 150                                   | 26  |
| MRA         | spironolactone 25-50 (eplerenone)                              | 6   |
| SGLT2i      | dapagliflozin / empagliflozin 10                               | 22  |

~73% combined mortality reduction at target doses; only ~1% reach them (CHAMP-HF). **Add/titrate something every visit** (q1-2 wk; STRONG-HF).

## Device / vasodilator (EF<35%, ≥3 mo GDMT)

- **CRT** if QRS >120-150 ms, esp. LBBB — NNT 8
- **ICD** if life expectancy >1 yr (CRT-D combines both) — NNT 23
- **Hydralazine + isosorbide**: self-ID Black pt on GDMT (Class 1a, A-HeFT) or when ACEi/ARB/ARNI not tolerated

## 3 · Across the EF spectrum

| Category | Therapy                                             |
|----------|-----------------------------------------------------|
| HFimpEF  | Continue GDMT <b>indefinitely</b> (TRED-HF relapse) |
| HFmrEF   | SGLT2i; HFrEF-style per EF/trajectory               |
| HFpEF    | SGLT2i (2a US / <b>1A ESC 2023</b> ); MRA/ARNI 2b   |

**New since 2023**: finerenone FDA-approved for LVEF ≥40% (2025, FINEARTS-HF); treat obesity → GLP-1/tirzepatide improve HFpEF outcomes (STEP-HFpEF, SUMMIT). *Full attending dotphrase is in the HTML lesson package.*

## When to start each pillar

- **BB**: only when compensated (euvolemic).
- **ARNI**: SBP>100, 36h off ACEi (ARB→immediate); can start inpatient.
- **ACEi/ARB**: stable Cr & K.
- **MRA**: hold if K>5, GFR<30, Cr>2.5 M />2 F.
- **SGLT2i**: avoid GFR<20, type 1 DM / DKA hx.